Neurosurgery Consultation Note

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Case ID	SYN-0020	**Age / Sex**	74 / M
Setting	Neurosurgery ward	**Primary dx**	Chronic subdural hematoma

Reason for consultation

Progressive confusion, gait unsteadiness, and headache for 3 weeks.

History of present illness

Minor head bump on a cupboard door about 6 weeks ago, no medical review. Family reports slowing, falls (2 in the last week), word-finding problems, and sleeping much of the day. No vomiting or seizures.

Past history / medications

Hypertension, ischemic heart disease with aspirin 100 mg daily, moderate alcohol intake.

Examination

BP 150/85, HR 72. Mild right arm drift, slowed responses, wide-based gait. Pupils equal, reactive. No papilledema. Oriented to person but not date.

Glasgow Coma Scale

Eyes open spontaneously. Speaks in sentences but is confused regarding time and place. Obeys commands.

GCS on consultation	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	4	4 - Confused
Motor (M)	6	Obeys commands
TOTAL GCS		**14**

GCS post-operative day 2	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	5	5 - Oriented
Motor (M)	6	Obeys commands
TOTAL GCS		**15**

Investigations

CT head: left hemispheric crescentic hypodense subdural collection, 22 mm maximum thickness, midline shift 9 mm, effaced sulci. Coagulation normal. Platelets 245.

Assessment

Symptomatic chronic subdural hematoma with mass effect, on aspirin. GCS 14.

Plan / disposition

Aspirin held. Burr-hole evacuation with subdural drain under general anesthesia, performed without complication. Post-operative CT: marked reduction of collection and shift. GCS 15 on day 2 with improved gait. Discharged to physiotherapy, clinic review at 6 weeks.